

إسم المريض ربيعاً : جيهان مرشد حنا بشاي
المحافظة : القاهرة
رقم البطاقة : 27811230100829 رقم التليفون : 01222395001
العمر(تاريخ الميلاد) : 23-11-1978 المهنة : ربه منزل
العنوان : ++++++

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-
الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----) المقر بما فيه
التوقيع: (-----)

التعليق على التشخيص : -----

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

التشخيص النهائي :
 خطة العلاج :
 المستشفى : قسم :
 جهة العلاج : نفقات :
 ملاحظات :

**رئيس
المجلس الطبي المتخصص**

--

Name: جيهان مرشد حنا بشاي National ID: 27811230100829	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Capecitabine therapy

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



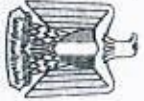
٢٠٢٤/٠٢ ٢٧٨١١٢٣٠١٠٠٨٢٩

حاصله على دبلوم تجاره

انثى مسجدة

عداد عوض مكسيوس ايوب

البطاقة سارية حتى ٢٠٣١/٠٢/٠٩





جهاز شؤون الموظفين

بطاقة تحقيق المساهمة

جيهان

مرشد حنا بشاي

٣٢ من القيسه المرقسيه

الاربعه - القاهره

٢٧٨١١٢٣٠١٠٠٨٢٩




KJ9363956

نسخة



دلتا
للأشعة

مركز

Patient Name: جيهان مرشد حنا	Ref. Phy.: وزارة الصحة والإسكان
Study Date: 25/03/2026 01:20:00 م	Acc No: 420397
Patient ID: 261184	

MRI OF THE PELVIS & BOTH HIPS

TECHNIQUE:

- Sagittal T2WIs
- Axial T1 and T2 WI
- Coronal T1, T2 and STIR WI

FINDINGS:

- Normal marrow signal intensity pattern of both femoral heads, rest of upper femora and scanned pelvic bones. No evidence of marrow oedema or infiltration.
- Intact both hip joints with no joint effusion or synovial thickening.
- Normal signal and comparable bulk of the different muscle groups around both hip joints.
- Preserved fascial fat planes.

OPINION:

- Normal study of the bony pelvis.

Dr. Nagy Mohamed, MD
Radiology consultant

فرع شبين القناطر
و وحدة الرنين المغناطيسي برج الياسمين موقف مسطرد
و وحدة الأشعة المقطعية - شارع بورسعيد بجوار ستوديو ميامي
0132711199 01273339932 - 01200043245

فرع شبرا الخيمة
و شارع ١٥ مايو - محطة المرور - بجوار البتلان الأهلى وبتلك قطر الوطنى
01281130055 02-42222279 01030711113
و فرع مسطرد: ٤ ش ١٥ مايو - مسطرد (شبرا الخيمة تالى)
02-44731775 01287000024

Patient Name: جيهان مرشد حنا	Ref. Phy.: وزارة الصحة والامكان
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فرع شبين القناطر

وحدة الرنين المغناطيسي برج الياسمين موقف مسطرد
وحدة الأشعة المقطعية - شارع بورسعيد بجوار ستوديو ميامي
0132711199 01273339932 - 01200043245

فرع شبرا الخيمة

شارع ١٥ مايو - محطة المرور - بجوار البنان الأهلي وبنان قطر الوطني
01281130055 02-42222279 01030711113
فرع مسطرد: 1 ش ١٥ مايو - مسطرد (شبرا الخيمة ثاني)
02-44731775 01287000034

Name : Jihan Morshed Hana Bishay
Branch : Portable
Ref.Physician Prof : cap+c

Date : 19-02-2026 10:22
Medical No. : GRI-27811230100829
Ins.Company :

- stones or back pressure changes. Left upper renal calyceal stone measures about 6.7x4.3 mm with attenuation value about 857 HU
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
 - The urinary bladder shows no evidence of gross masses or diverticulae or stones.
 - Normal appearance of the pelvic organs with no pelvic masses or collections.
 - Bilateral prominent inguinal lymph nodes, being evident on left side measures about 9 mm.
 - No ascites.
 - Subcutaneous fatty cyst measures about 4.3x4.8 cm
 - The examined skeleton bilateral iliac sclerotic foci for bone scan

Opinion:

- No pulmonary or hepatic deposits.
- Left upper renal calyceal stone,
- Bilateral iliac bones sclerotic foci for bone scan.

Dr.Norhan Ramzy,MD

Dr.NOURHAN RAMZY

Dr.NOURHAN RAMZY

Name : Jihan Morshed Hana Bishay

Branch : Portable

Ref.Physician Prof : cap+c

Date : 19-02-2026 10:22

Medical No. : GRI-27811230100829

Ins.Company :

**MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST,
ABDOMEN AND PELVIS REVEALED:**

A known case of right breast cancer with no available clinical data.

I- CHEST:

Compared to the previous CT report dated 19/8/2025, the current examination revealed

- **Rather stationary course** of the previously noted right breast LIQ soft tissue mass lesion measures about 6.5x6.5 cm in its TS,AP dimensions respectively with overlying skin thickness measures about 1.3 cm. it is seen indenting the underlying pectoralis muscle.
- Another soft tissue lesion is seen adherent to the forementioned lesion it measures about 3x2.6 cm
- **Still noted** heterogenous parenchymal texture of the right breast associated with ipsilateral enlarged axillary lymph nodes, the largest measures about 3x2 cm for further sonomammography.
- **Still noted** middle lobar subpleural atelectatic bands likely post treatment sequel.
- No pathological pulmonary nodules, masses or cavitary lesions.
- Still noted prominent mediastinal lymph node , the largest is seen right paratracheal measures about 13mm.
- No pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.
- Small bilateral fibroelastoma dorsi.

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- The gall bladder is well distended, average wall thickness with no stones.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No right

Name : Gihan Murshid Hana Bishay

Branch : Dokki

Ref. Physician Prof : Gostaf Roseh Hospital

Date : 06-10-2025 11:47

Medical No. : 219135

Tc^{99m} MDPSKELETAL SCINTIGRAPHY**History**

- A 47-year-old patient, with history of breast cancer and referred for metastatic workup.

Technique

- Anterior and posterior whole-body scans were acquired 2 hours post injection of the tracer.

Findings

- No abnormal active or cold focal bony lesions that could account for metastatic bone disease all over the surveyed skeleton.
- Degenerative changes are noted at the spine and big joints.

Impression:

- The skeleton is free from metastases.

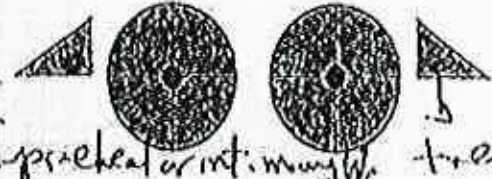
Best regards,
Dr. Alaa Nagi, MD
Nuclear Medicine

MDT FORM

22/10/2025 G

Name	صبا محمد شحات	MRN	CV1112101000009
Age	46	ID	1A7A/C0
Menopausal status	regul	Phone	01222-395001
NRS SCORE		Marital status	
BMI		Family history	-VC-
CO morbidity	Free	REASON OF MDT	

Clinical Data

BX:	INC 6 III	R	L
ER:	718	PR:	718
HER 2:	+3	KI67:	70%
Sonomammography: Rt lower & mass 7x5cm diff & skin thickening, another mass 6x3cm +50' chest, Rt pathology 11/17/23, No supraclav or int. lymph			
Mets Workup: U enlarged axly LN, thickened Cortex, proven hdx		TNM cl 2 traced lymph reactive nodes +50cm	
CTRAP → free except for sclerotic foci bone scan → free			

Recommendations:

NA TCWP X 6 cycles
Rn MRM

W2, W3
13/11/25

MDT OUTCOME & PLAN

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
M. K. K.					

PATHOLOGY DEPARTMENT HISTOPATHOLOGICAL REPORT

Patient name: جيهان مرشد حنا بشاي

Age: 47 years

Sex: Female

Request type: مبالغة

Sub request type:

Hospital no:

Pathology No.: 1968-2025

Department: صحة المرأة

Date of result: 2/5/2025

Referred by: د/مروة

Receiving date: 2/5/2025

Nature of specimen: Tru-cut Biopsies.

I.D. Number: 27811230100829

Clinical diagnosis: Right UIQ breast mass BIR: OS US Left axillary indeterminate lymph nodes.

Gross pathology:

- 1- Right breast mass: Multiple grayish white tissue cores measured up to 1.5 cm long, totally submitted.
- 2- Left axillary lymph node: Multiple grayish white tissue cores measured up to 1 cm long, totally submitted.

Microscope picture:

- 1- Section examined revealed cores of breast tissue showing invasive duct carcinoma of no special type (NST). The tumor forms sheets of anaplastic, nuclear cells infiltrating among desmoplastic stroma. Tumor cells showed moderate anaplasia and poor tendency to form tubules. There is high mitotic activity. Areas of necrosis are seen. No DCIS.

ER: Strong positive nuclear staining in 60% of tumor cells (Allred score 7/8).

PR: Strong nuclear staining in 40% of tumor cells (Allred Score 7/8)

Her2: Strong circumferential membranous staining in more than 10% of tumor cells, Score (3+).

Ki67: positive nuclear staining in 70% of tumor cells.

2. Section examined revealed cores of lymphoid tissue showing prenat sinuses & formed of reactive follicles. No evidence of metastatic deposits.

Diagnosis:

- 1- Right breast mass, Tru-cut biopsy: Invasive Duct Carcinoma, Grade III. (ER: Positive, 7/8), (PR: Positive, 7/8), (Her2: Positive, Score 3+), (Ki67: 70%).
- 2- Left axillary lymph node, Tru-cut biopsy: Reactive nodal tissue, Negative for metastatic deposits.

Signature:

Dr. Nermin Nabil, M.D.

Pathology Lab. Manager

Nermin Nabil, M.D.



مركز نوبية للاشعاع والميكرو
ومعالجة سرطان الثدي

T
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Patient Name: GEHAN MURSHIED HANA DASHAY	Gender: Female
MAN: 20225021902	Ordered By: Aya abdallah aboelftough mohamed
DOB: 23/11/1978 Age: 46 Y 9 M 1 D	Order ID: 228529

2D mammography

Procedure Date: 02/08/2025 12:42:00 PM

Report:

Report:

FULL FIELD DIGITAL MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND

INDICATION:

- Right breast lump.

TECHNIQUE:

- CC and MLO view.
- Complementary ultrasound

REPORT:

- Scattered fibroglandular densities (ACR II).
- Right breast shows:
- Right lower quadrant spiculated dense mass lesion measuring about 7x 5 cm, diffuse skin thickening, Complementary Ultrasound which showed right 4-7 o'clock hypoechoic vascular mass lesion reaching the skin, abutting underlying muscle.
- Another rounded lesion is seen at 5 o'clock measuring 1 x 1 o'clock, 1.9 cm away, medial and inferior to the main lesion.
- Right diffuse skin thickening reaching 1 cm.
- Right Pathological axillary lymph nodes (more than 3).
- No pathological supraclavicular, internal mammary lymph nodes.
- Left breast shows:
- No spiculated mass lesions or grouped microcalcifications.
- Normal skin thickness and contour.
- Normal nipple areola complexes.
- Left enlarged axillary lymph nodes, thickened cortex and preserved hila likely reactionary.

OPINION:

- Right locally advanced malignant mass lesions as described for histopathology (BIRADS5).
- Right pathological axillary lymph nodes.
- Left normal breast study (BIRADS 1)
- Left reactionary axillary lymph nodes for follow up.

Approved by: Marwa Omar, MSc

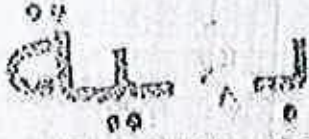
Date: 02/08/2025 Time: 13:27

Impression:

Recommendation

Print Date: 24/08/2025 12:30:17 PM

Page: 1/2



مركز أبحاث الأمراض المعدية
ومعالجة سرديات الفيروسات

T
F
W
E

Patient Name: GEHAN MURSHIED HANA BASHAY	Gender: Female
MRN: 20225021902	Ordered By: Aya abdallah aboelftouh mohamed
DOB: 23/11/1978 Age: 46 Y 9 M 1 D	Order ID: 228529

Recommendation:

Referral to clinic

Approved by: Marwa Omar, MSc

Date: 02/08/2025 Time: 13:27



Patient's Name: Gihan Murshid Hana Beshay
Date: 2026-08-24
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

CT CHEST, ABDOMEN AND PELVIS WITH CONTRAST

CLINICAL INDICATION:

Known right breast carcinoma subjected to RT.

TECHNIQUE:

Contrast-enhanced CT examination of the chest, abdomen, and pelvis was performed following intravenous administration of iodinated contrast material. Multiplanar reformatted images were obtained.

FINDINGS:

CHEST:

There is a large irregular heterogeneous soft-tissue mass involving the right breast, compatible with the known breast malignancy, measuring $10.8 \times 8.5 \times 7.6$ cm with overlying skin thickening and retracted nipple. The lesion is overlying the ribs with no osseous infiltration.

Multiple enlarged bilateral axillary lymph nodes are identified, demonstrating suspicious morphology, with the largest on the right axilla, measuring 1.8×9 cm, concerning for metastatic nodal involvement.

No focal pulmonary mass or suspicious pulmonary nodules are identified.

No pleural effusion or pneumothorax.

No focal chest wall invasion or destructive osseous lesion is identified on CT.

ABDOMEN AND PELVIS:

The liver is normal in size and enhancement, with no focal suspicious hepatic lesion.

The gallbladder and biliary tree are unremarkable.

The spleen, pancreas, and adrenal glands show no focal suspicious abnormality.

Both kidneys demonstrate preserved symmetric enhancement, with no hydronephrosis or suspicious focal lesion.

No pathologically enlarged abdominal, retroperitoneal, or pelvic lymph nodes.

No ascites, peritoneal nodularity, or omental deposits.

No bowel obstruction.

The urinary bladder is unremarkable.

No suspicious pelvic organ abnormality is identified.

No definite suspicious lytic or sclerotic osseous lesion is identified in the visualized skeleton.

IMPRESSION:

1. Large right breast mass, consistent with the known breast carcinoma.
2. Right axillary lymphadenopathy, suspicious for metastatic nodal involvement.
3. No CT evidence of distant metastatic disease in the chest, abdomen, or pelvis.
4. No suspicious pulmonary, hepatic, peritoneal, or osseous metastatic lesions identified.

جوسافي
رئيسة
رئيسة

q-11



مبادرة رئيس الجمهورية
لحماية صحة المرأة المصرية

الجمهورية
مصرية
مصر

09/13/2026

MDT FORM

Name:	Jihan Morshed Hna	MRN:	4130
Age:	48 Y	ID:	27811230100829
Menopausal Sta		Phone:	01278167983
NRS SCORE:		Marital Status:	
BMI:		Family History:	
CO Morbidity:		Reason Of MDT:	

Clinical Data:

BX:

R

L

ER:

PR:

HER2:

HER2:

KI67:

Sonomammography:

Mets Workup:

History:

prem rt breast cancer ct4d n2 m0 idc 3 er 7/8 pr7/8 her23+ rec 3 taxotere carbopl.
pt rec 6 cycles taxotere carboplatin then 9 weeks of taxol carbo and 6 phesgo
still DP with no mets
CT CAP >> STILL NO METS LOCALLY ADVANCED BREAST CANCER

T N M

Recommendations:

for xeloda /tykerb

MDT Outcome & Plan

xeloda /tykerb

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د. مهنى عزت	د. أحمد نجيب	د. مهنى عزت			